

Policy on Credentialing and Privileging of Para-Clinical Professionals

«Hospital Name»

Document control

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Prepared by	HR In-Charge / Personnel Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» runs a process for credentialing and privileging of para-clinical professionals permitted to provide patient care without supervision, so that only identified, qualified, and privileged professionals provide unsupervised care.

1. Purpose

This policy sets out how «Hospital Name» identifies para-clinical professionals permitted to provide unsupervised patient care, documents and verifies their credentials, grants and communicates their privileges, and ensures they practise within those privileges.

This policy does not cover recruitment, induction, safety and quality training, or staff health and safety — those are covered in other hospital policies.

2. Scope

This policy applies to all para-clinical professionals at «Hospital Name» — for example physiotherapists, rehabilitation therapists, dieticians, pharmacists, clinical pharmacists and technologists — permitted by law, regulation and the organisation to provide patient care without supervision.

3. Policy standards

«Hospital Name» identifies para-clinical professionals permitted to provide patient care without supervision.

«Hospital Name» documents and periodically updates the education, registration, training and experience of para-clinical professionals.

«Hospital Name» grants para-clinical professionals privileges in consonance with their qualification, training, experience and registration.

«Hospital Name» ensures the requisite services a para-clinical professional is authorised to provide are known to that professional and to relevant departments.

«Hospital Name» ensures para-clinical professionals care for patients as per their privileging.

4. Non-negotiable rules

1. Do not let a para-clinical professional provide unsupervised patient care without first identifying them on the permitted list.
2. Do not leave a para-clinical professional's education, registration, training or experience unverified, undocumented, or out of date.
3. Do not grant or allow privileges that are not in consonance with a para-clinical professional's qualification, training, experience and registration.
4. Do not leave a para-clinical professional or the relevant departments unaware of the services that professional is authorised to provide.
5. Do not let a para-clinical professional care for a patient outside their granted privileges.
6. Do not bypass the stop-work authority in Section 6 when the trigger conditions are met.
7. Staff who see a rule under this policy broken report it the same shift to the HR In-Charge / Personnel Officer or the Medical Superintendent.

5. What we do

5.1 Identification of permitted professionals

«Hospital Name» identifies para-clinical professionals (for example physiotherapist, rehabilitation therapist, dietician, pharmacist, clinical pharmacist, technologist) permitted by law, regulation and the organisation to provide patient care without supervision — individuals with the required qualification(s), training and experience, in consonance with the law. Providing unsupervised care outside this identified list is a stop-work trigger (Section 6).

5.2 Credential documentation and verification

The education, registration, training and experience of para-clinical professionals at «Hospital Name» are appropriately verified, documented and updated periodically, after acquisition of new skills or qualification, by checking with the organisation that awarded the qualification or training.

5.3 Granting of privileges

«Hospital Name» grants para-clinical professionals privileges in consonance with their qualification, training, experience and registration — specifying what each is authorised to do,

with the requisite registration or licence held where applicable. Granting or exercising privileges outside this record is a stop-work trigger (Section 6).

5.4 Communication of privileges

The requisite services a para-clinical professional at «Hospital Name» is authorised to provide are known to that professional and to the concerned departments, communicated internally.

5.5 Practising within privileges

Para-clinical professionals at «Hospital Name» care for patients as per their privileging. New staff may work under supervision until independent privileges are granted; the hospital maintains a mechanism confirming para-clinical professionals provide only the services they are privileged for.

6. Stop-work authority

Do not let a para-clinical professional provide patient care without supervision unless the HR In-Charge / Personnel Officer and the credentialing file show that professional identified and privileged for that care under this policy.

Stop-work applies to starting or continuing unsupervised care by that professional. Care already under way is handed to a privileged professional or brought under supervision; immediate life-saving care is not withdrawn while that handover happens.

The person who stops tells the HR In-Charge / Personnel Officer and the Medical Superintendent the same shift. Refusing to let an uncredentialed or unprivileged professional practise unsupervised is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
HR In-Charge / Personnel Officer	Owns this policy. Maintains the credentialing file and the identified-professional list.
Department heads / Unit In-Charges	Verify credentials and confirm para-clinical professionals practise within their granted privileges.
Medical Superintendent	Oversees implementation of this policy. Receives stop-work escalations.
All Staff	Report any para-clinical professional providing unsupervised care outside the identified or privileged list.

8. Quality monitoring

The HR In-Charge / Personnel Officer reviews credentialing files to confirm every para-clinical professional providing unsupervised care is identified and privileged. Documentary evidence is

on file for each CORE objective element. Compliance with the stop-work authority in Section 6 is monitored through the credentialing file review. This policy itself is reviewed every year.

9. Training and staff acknowledgement

All staff covered by this policy are made aware of the credentialing and privileging process, including the stop-work authority in Section 6, and sign the acknowledgement below.

I have read the Policy on Credentialing and Privileging of Para-Clinical Professionals of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

HR department; all department heads / unit in-charges; all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HR	Human Resources
HRM	Human Resource Management (NABH Hospitals 6th Edition chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition HRM.13

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required. Stop-work applies ONLY to the OEs marked 'Section 6 Stop-work' below.

OE	Level	Requirement	Where addressed	Responsible
HRM.13.a	CORE	Para-clinical professionals permitted by law, regulation and the organisation to provide patient care without supervision are identified.	Section 3; 5.1; Section 6 Stop-work	HR In-Charge / Personnel Officer
HRM.13.b	Commitm	The education, registration,	Section 3; 5.2	HR In-

	ent	training and experience of para-clinical professionals are appropriately verified, documented and updated periodically.		Charge / Personnel Officer
HRM.13.c	CORE	Para-clinical professionals are granted privileges in consonance with their qualification, training, experience and registration.	Section 3; 5.3; Section 6 Stop-work	HR In-Charge / Personnel Officer
HRM.13.d	Commitment	The requisite services to be provided by the para-clinical professionals are known to them as well as the concerned departments/units of the organisation.	Section 3; 5.4	HR In-Charge / Personnel Officer
HRM.13.e	Commitment	Para-clinical professionals care for patients as per their privileging.	Section 3; 5.5	HR In-Charge / Personnel Officer

13. Required Records / Evidence Checklist

HRM.13.a — Permitted para-clinical professionals identified. [Stop-work trigger]

- Identified-para-clinical-professional list permitted to provide unsupervised patient care, with a qualification, training and experience verification record.
- Cross-reference to the stop-work authority in Section 6 for unlisted practice.

HRM.13.b — Credentials verified, documented, updated periodically.

- Education, registration, training and experience verification and documentation record, updated periodically, with a personal-file record of each update.
- Verification-with-awarding-organisation record.

HRM.13.c — Privileges granted in consonance with qualification, training, experience and registration. [Stop-work trigger]

- Granted-privilege record naming what each para-clinical professional is authorised to do, with a registration or licence-on-file record where applicable.
- Cross-reference to the stop-work authority in Section 6 for privileging outside this record.

HRM.13.d — Requisite services known to professional and relevant departments.

- Communicated-service record to the professional and to concerned departments, with an internal-communication and confirmation-of-awareness record.

HRM.13.e — Para-clinical professionals care as per privileging.

- Supervision record for new staff until independent privileges are granted.
- Mechanism-confirmation and privileging-compliance spot-check record.

14. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard HRM.13.
- NABH Guidebook to NABH Accreditation Standards for Hospitals, 6th Edition — HRM.13 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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